

THE PCOR-ANZ ANNUAL REPORT 2023



FUNDING & ENDORSEMENTS

MOVEMBER®

PCOR-ANZ is principally funded by Movember, primarily in partnership with:



ACT Health



Canberra Health Services



MENZIES+

Cancer Institute NSW
cancer.nsw.gov.au



CHOMNZ | centre for health outcome measures new zealand



PCOR-ANZ is endorsed by:



Please refer to each jurisdiction's website for a full list of contributing organisations.

CONSUMER VERSION OF THE ANNUAL REPORT

Today:

- We are going to present to figures from the recently released PCOR-ANZ 2023 Annual Report
- The PCOR-ANZ Annual Report summarises information on
 - Acknowledgements, funding and endorsements
 - Purpose and aims of PCOR-ANZ
 - Population characteristics
 - Diagnosis of prostate cancer
 - Management of prostate cancer
 - PROMs for people with prostate cancer
- The Annual Report audience is intended for mostly for clinicians
- This year, we want to develop a **Consumer Version** of the Annual Report intended for the men with prostate cancer audience



THE PCOR-ANZ ANNUAL REPORT 2023

CONSUMER VERSION

AUDIENCE

- Public / Lay Person

LENGTH

- Max 10 pages

STYLE

- Simple informative messages and infographics which educates the reader about:
 - Purpose of PCOR-ANZ
 - Highlights the key findings from the Annual Report that **are relevant and interesting to men with prostate cancer, their families and carers**



ABOUT THIS REPORT

WHAT DATA IS INCLUDED IN THE REGISTRY?

Jurisdictional PCOR collect a minimum clinical dataset relating to the diagnosis, treatment of prostate cancer as well as patient-reported outcomes.

DIAGNOSIS

- Transrectal ultrasound guided biopsy
- Transperineal biopsy
- Transurethral resection of the prostate

CANCER INFORMATION

- Cancer stage
- Gleason score
- Cancer risk category
- Prostate-specific antigen (PSA) levels

TREATMENT

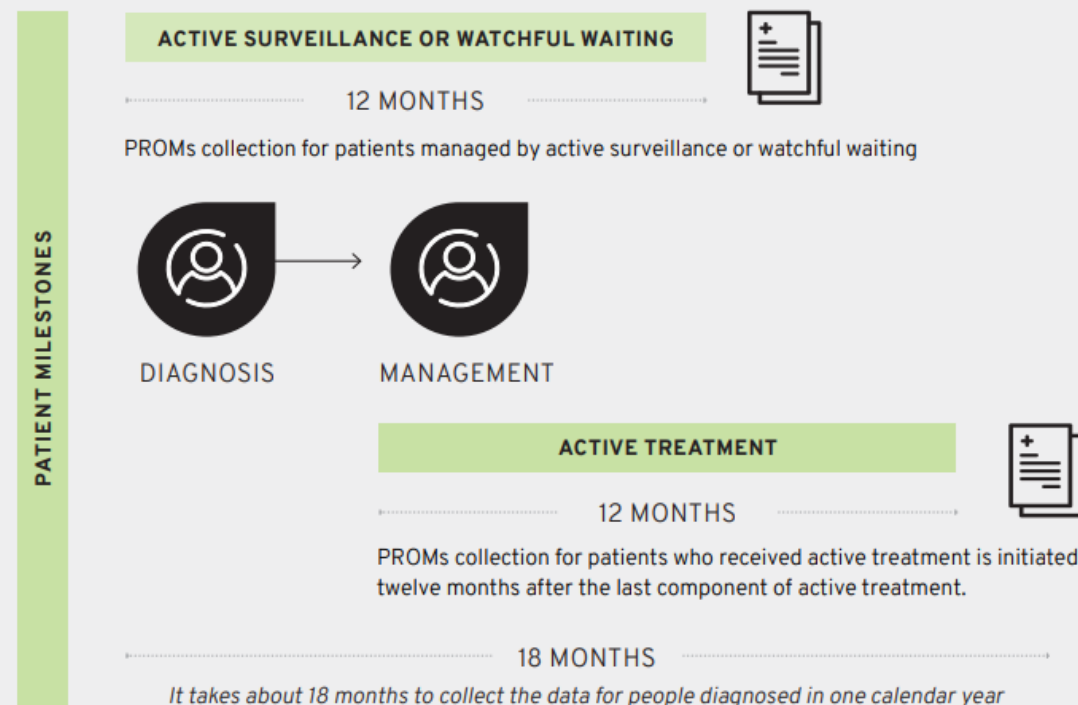
- Treatments provided (e.g. surgery, radiotherapy, chemotherapy, androgen deprivation therapy)
- Whether active surveillance or watchful waiting protocols were followed
- Disease progression

PATIENT-REPORTED OUTCOME MEASURES (PROMs)

Functional outcomes using the symptom questionnaire (EPIC-26) and questions on libido and use of sexual aids:

- Twelve months following diagnosis for active surveillance or watchful waiting; or
- Twelve months after start of active treatment

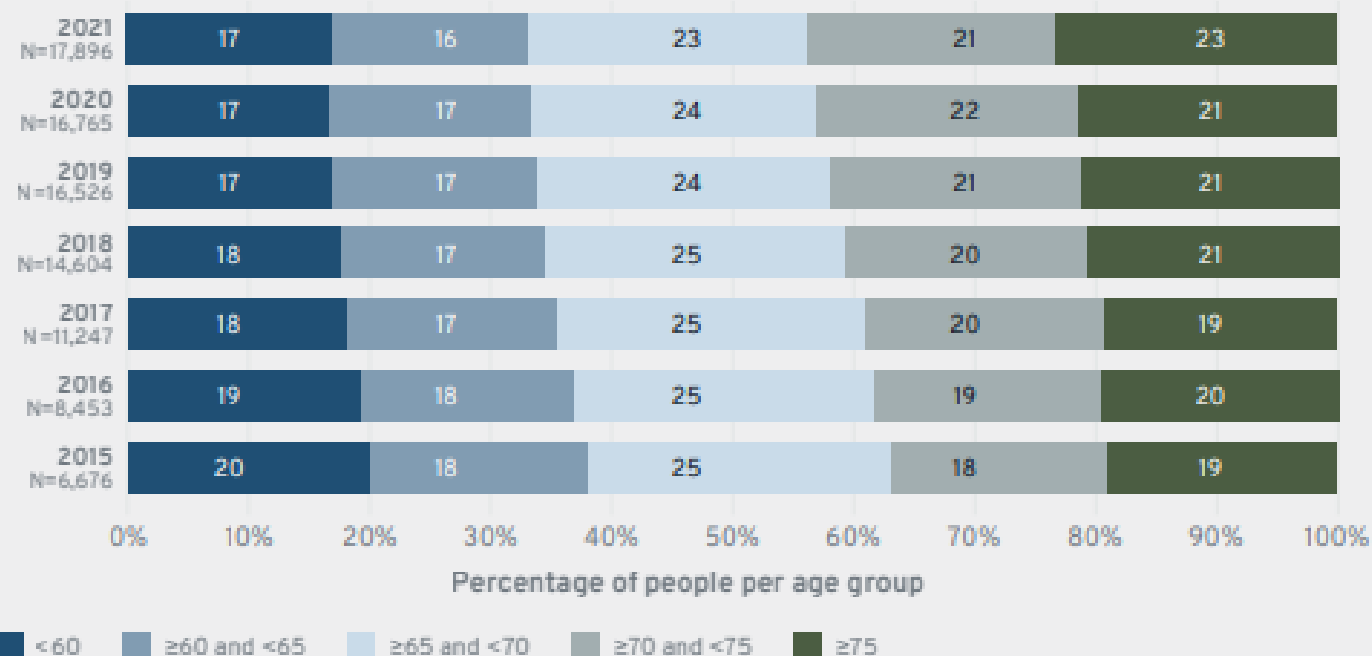
WHEN DO WE COLLECT INFORMATION?



POPULATION CHARACTERISTICS

- Changes in the age of men who are diagnosed with prostate cancer over time

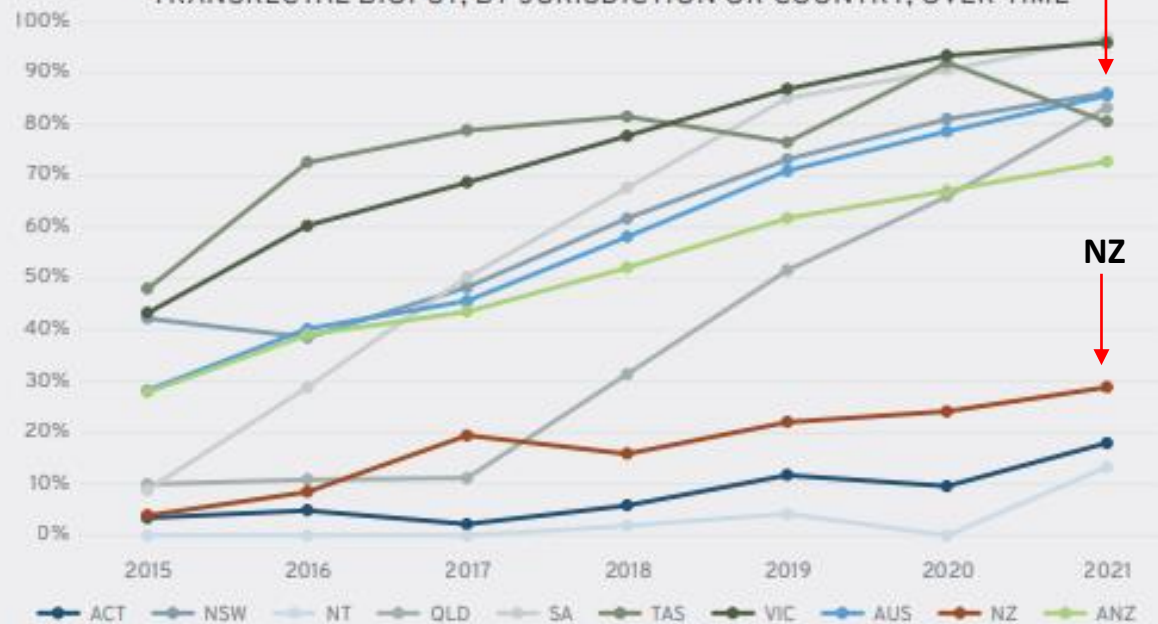
FIGURE 2: AGE GROUP AT DIAGNOSIS PER YEAR AS A PERCENTAGE OF TOTAL PEOPLE REGISTERED IN EACH YEAR



- Proportions per age group at diagnosis are calculated as a percentage of total PCOR-ANZ registrants with available age group data (from all jurisdictions combined) in each diagnosis year.
- Data on age group was available for 100% (92,167/92,167) of people in the database.
- See Table S3 for information on mean and median age at diagnosis.
- Percentages are rounded and may not add to 100%.

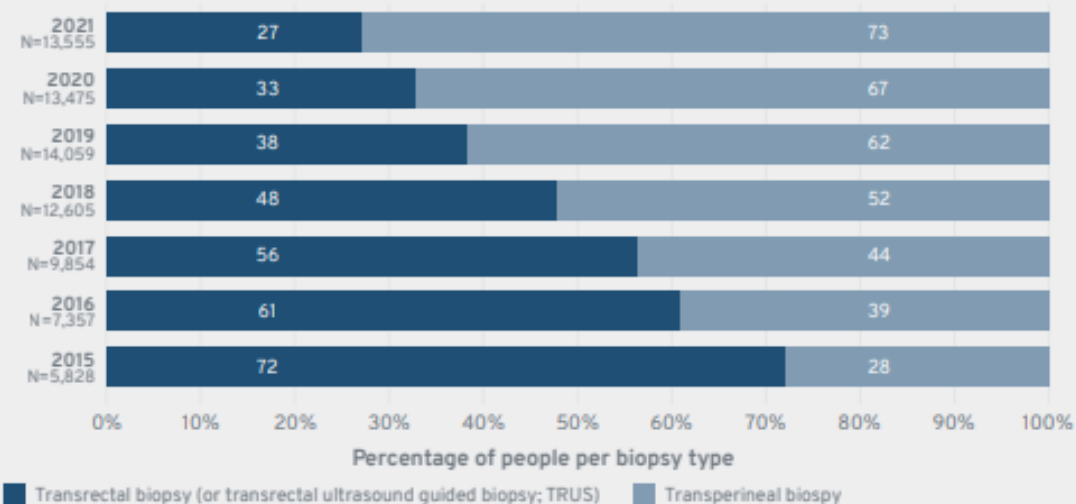
DIAGNOSIS OF PROSTATE CANCER

FIGURE 9: PROPORTION OF PEOPLE DIAGNOSED BY TRANSPERINEAL VERSUS TRANSRECTAL BIOPSY, BY JURISDICTION OR COUNTRY, OVER TIME



- Percentage of transperineal biopsy per year was calculated as a percentage of PCOR-ANZ registrants in each jurisdiction who were diagnosed by either transrectal or transperineal biopsy. People diagnosed by other methods were not included in this analysis.
- Information on method of diagnosis was available for 97.6% (89,957/92,167) people in the database; of whom 76,733 were diagnosed by either transperineal or transrectal biopsy (83.3% of all registrants).
- Percentages are rounded and may not add to 100%.
- See Table S6 for more information on diagnosis method per jurisdiction or country.

FIGURE 8: PROPORTION OF PEOPLE DIAGNOSED BY TRANSRECTAL VERSUS TRANSPERINEAL BIOPSY, BY YEAR



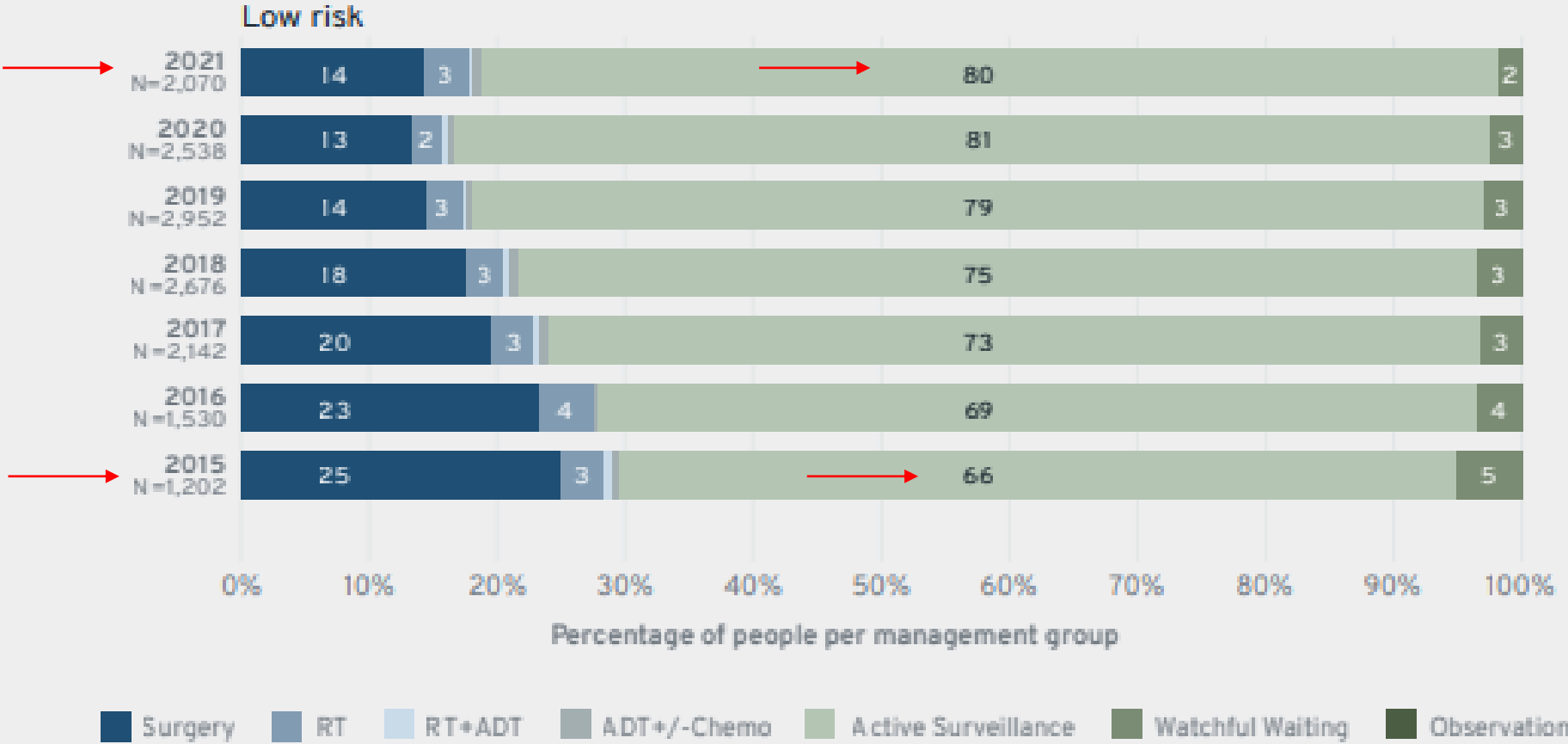
- Proportions per transrectal versus transperineal biopsy are calculated as a percentage of total PCOR-ANZ registrants who were diagnosed by either transrectal or transperineal biopsy (from all jurisdictions combined) in each year. People diagnosed by other methods were not included in this analysis.
- Information on method of diagnosis was available for 97.6% (89,957/92,167) people in the database; of whom 76,733 were diagnosed by either transperineal or transrectal biopsy (83.3% of all registrants).
- Percentages are rounded and may not add to 100%.

- Changes in the way a prostate biopsy is performed over time and across Australia and NZ

CHAPTER 3

MANAGEMENT OF PROSTATE CANCER

FIGURE 12: INITIAL MANAGEMENT PROVIDED BY NCCN RISK GROUP AND YEAR



CHAPTER 4

PROMS FOR PEOPLE WITH PROSTATE CANCER

What are PROMs?

- PROMs = Patient Reported Outcome Measures
- PCOR-ANZ uses *EPIC-26 questionnaire*
 - collects information from men with prostate cancer about certain outcomes or 'side effects'
- Across the PCOR-ANZ, the EPIC-26 questionnaire is filled out at 12 months after treatment
 - *Some jurisdictions also collect the questionnaire just after the point of diagnosis*

What is included in the EPIC-26 questionnaire?

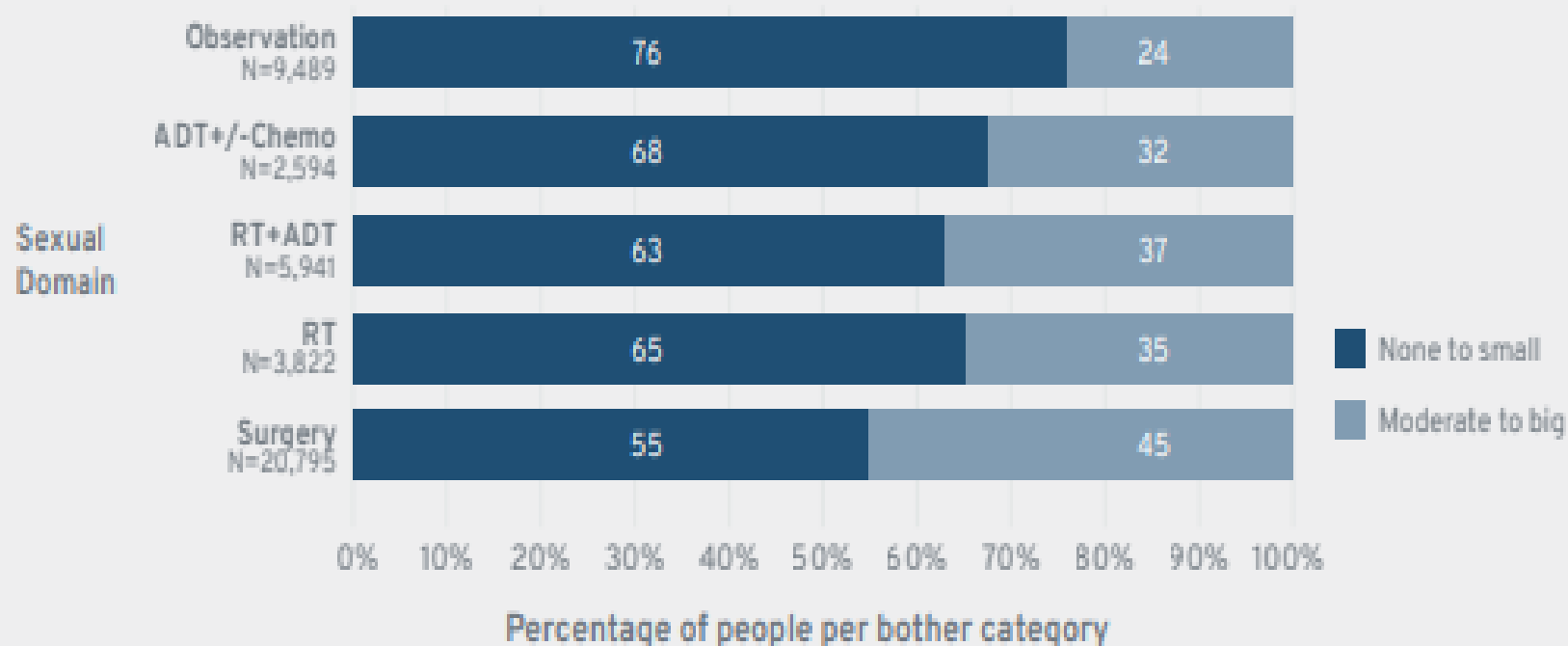
- Section on Bowel function
- Section on Sexual function
- Section on Urinary function
- Section on Hormonal function

PROMS FOR PEOPLE WITH PROSTATE CANCER

- Overall sexual bother/side effects experienced by men by treatment group

Overall, how big a problem has your sexual function or lack of sexual function been for you during the last 4 weeks?

FIGURE 17: PATIENT-REPORTED BOTHER 12 MONTHS AFTER INITIAL MANAGEMENT PROVIDED, BY FUNCTIONAL DOMAIN AND TREATMENT TYPE (2015-2021)

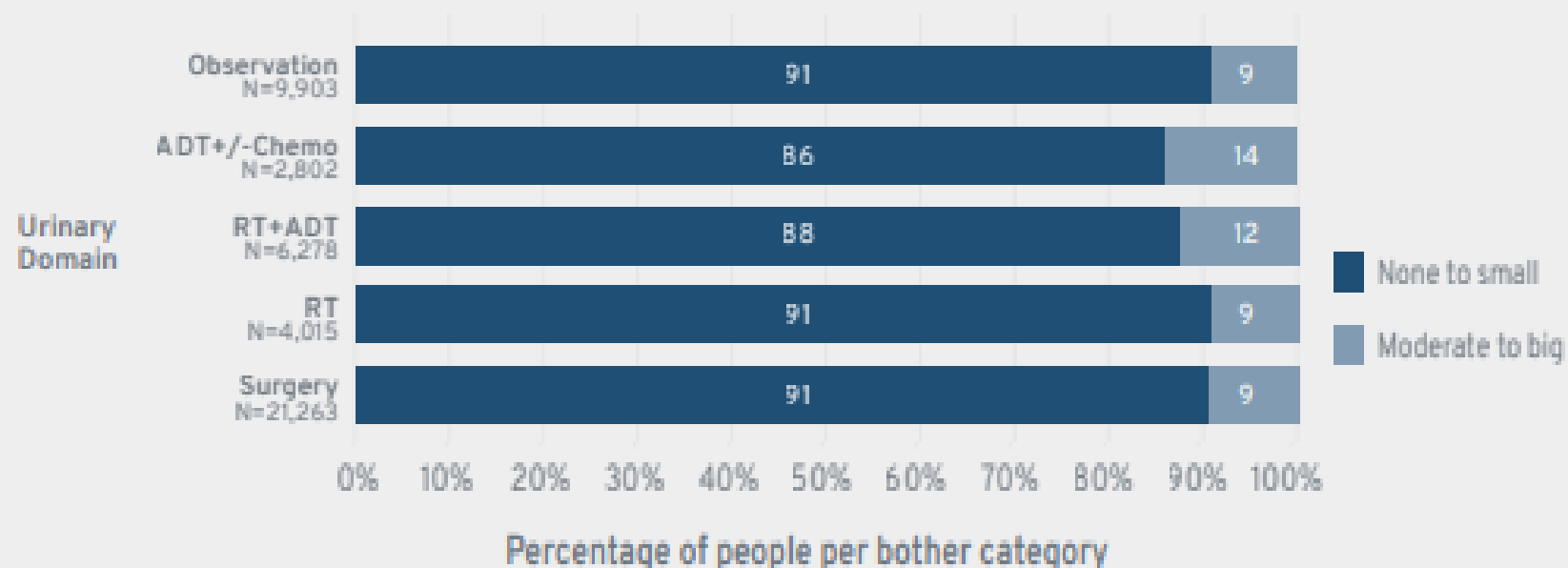


PROMS FOR PEOPLE WITH PROSTATE CANCER

- Overall urinary bother/side effects experienced by men by treatment group

Overall, how big a problem has your urinary function been for you during the last 4 weeks?

FIGURE 17: PATIENT-REPORTED BOTHER 12 MONTHS AFTER INITIAL MANAGEMENT PROVIDED, BY FUNCTIONAL DOMAIN AND TREATMENT TYPE (2015-2021)

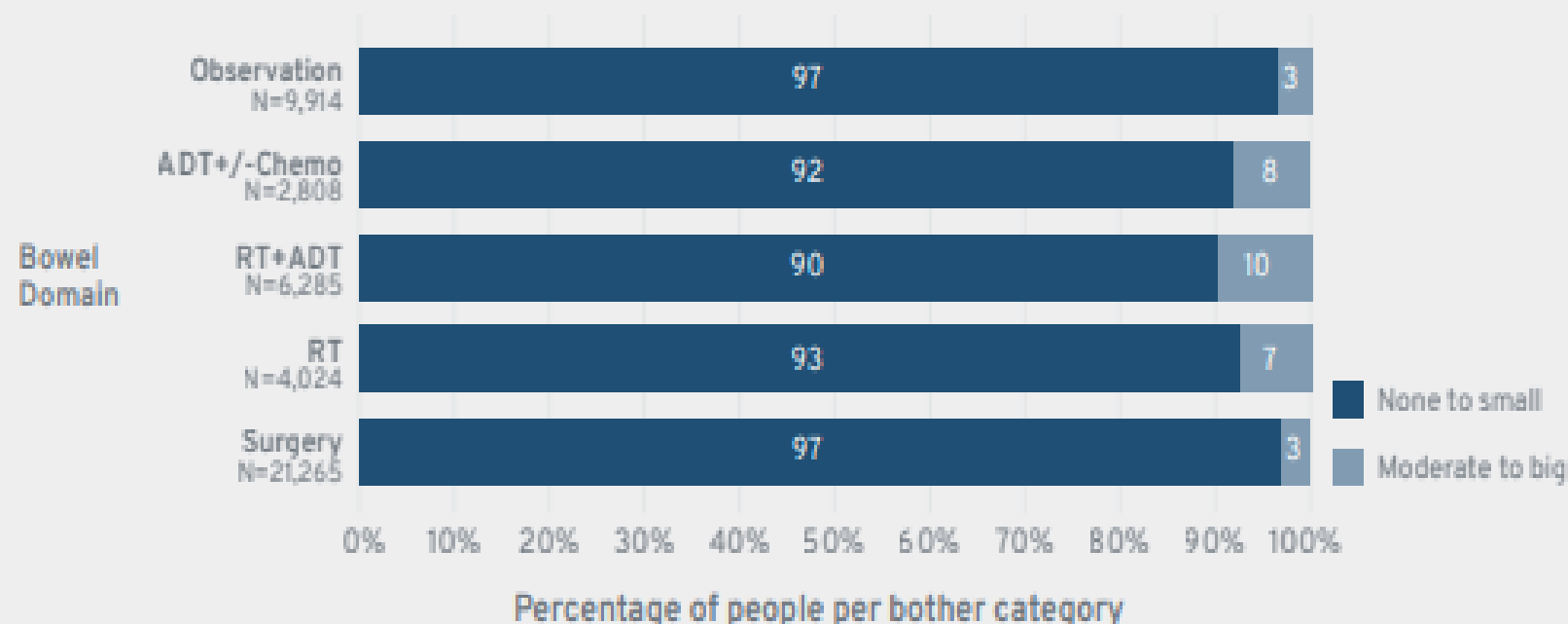


PROMS FOR PEOPLE WITH PROSTATE CANCER

- Overall bowel bother/side effects experienced by men by treatment group

Overall, how big a problem have your bowel habits been for you during the last 4 weeks?

FIGURE 17: PATIENT-REPORTED BOTHER 12 MONTHS AFTER INITIAL MANAGEMENT PROVIDED, BY FUNCTIONAL DOMAIN AND TREATMENT TYPE (2015-2021)



Examples from a project on Australian patient report cards for men with prostate cancer

Active surveillance

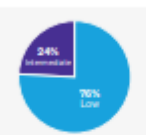
Active surveillance is an approach for managing prostate cancer by observation.

- It is only considered for men with low risk or very favourable intermediate risk prostate cancer.
- There is strong evidence that men with low risk prostate cancer can be safely managed through close monitoring.

Treatment is offered if there are signs that the cancer is progressing.

65yrs

Average age



Risk group

Prostate cancer survival

100%

5-year prostate cancer survival

>99%

10-year prostate cancer survival

100% of men who were on active surveillance had survived prostate cancer five years after diagnosis. This includes some men who switched to surgery or radiotherapy.

Number of deaths per 1000 men within 10 years of diagnosis



"I'm now five years since my diagnosis. I haven't experienced any problem because it was caught early"
- 73-year-old man who is on active surveillance

To know more about active surveillance, please visit pca.org.au/active-surveillance

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Surgery

Surgery involves removing the whole prostate gland, with all the prostate cancer. While it is an option for men in all risk groups, it is not usually an option for men whose prostate cancer has spread to other parts of the body.

64yrs

Average age



Risk group

Prostate cancer survival

>99%

5-year prostate cancer survival

99%

10-year prostate cancer survival

Number of deaths per 1000 men within 10 years of diagnosis



The chance of dying from prostate cancer increases across risk groups, but is still quite low after 10 years.

"I had worried too much about how long I could live but the high survival rate is something comforting and reassuring."

- 71-year-old man who had surgery

To know more about surgery, please visit pca.org.au/surgery

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External beam radiotherapy

External beam radiotherapy involves high energy X-rays being delivered to the prostate from outside the body to kill or damage cancer cells so they can't multiply or grow.

About 30% of men also received hormone treatment around the time of radiotherapy.

72yrs

Average age



Risk group

Prostate cancer survival

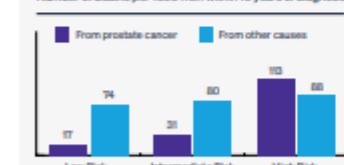
98%

5-year prostate cancer survival

95%

10-year prostate cancer survival

Number of deaths per 1000 men within 10 years of diagnosis



Generally, men with low and intermediate risk disease are more likely to die from causes other than prostate cancer. However, men with high risk prostate cancer are more likely to die from their cancer, rather than other causes.

"I am pretty much at risk of dying from other causes in the next 10 years than from prostate cancer"

- 70-year-old man who had radiotherapy

To know more about external beam radiotherapy, please visit pca.org.au/radiation

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PROPOSED CONTENT

Explanations (1 page)

- Risk groups
- Treatment types
- Patient reported side effects

Active surveillance/ watchful waiting (1 page)

- % of risk groups receiving this treatment
- % of men experiencing side effects for this treatment

Surgery (1 page)

- % of risk groups receiving this treatment
- % of men experiencing side effects for this treatment

RT with/without ADT (1 page)

- % of risk groups receiving this treatment
- % of men experiencing side effects for this treatment

ADT and chemotherapy (1 page)

- % of risk groups receiving this treatment
- % of men experiencing side effects for this treatment